Childbirth / Labor

AO-01 | Adult Obstetrics

Quick flow

01 Labor / Delivery Concern [EMT / AEMT / Paramedic]

Due date • gravida/para • contractions • membrane rupture • bleeding • fetal movement.

02 Maternal Emergency? [EMT / AEMT / Paramedic]

Severe bleeding • hypertension/hypotension • seizure • severe pain • altered mental status.

03 Inspect Perineum [EMT / AEMT / Paramedic]

No digital vaginal examination. Look for crowning, cord, breech, limb, or meconium.

04 Delivery Imminent? [EMT / AEMT / Paramedic]

Crowning, urge to push, frequent contractions, advanced parity, or unsafe transport.

05 Normal or Complicated? [EMT / AEMT / Paramedic]

Normal delivery • prolapsed cord • breech • shoulder dystocia • multiple gestation.

06 Mother + Newly Born Care [EMT / AEMT / Paramedic]

Control bleeding, maintain warmth, record times/APGAR, and notify destination.

Overview

- Determine whether transport is safe or delivery is imminent using gestational age, contraction pattern, urge to push, parity, prior labor duration, and perineal inspection.
- Prioritize maternal positioning, preparation for neonatal care, recognition of abnormal presentation, and immediate treatment of postpartum hemorrhage.

Provider-Level Actions

Perform immediately indicated care at the highest authorized level present while protecting both maternal and neonatal physiology.

EMT

- Place the mother left lateral when delivery is not imminent; document contraction frequency/duration and continuously reassess.
- For field delivery, support the head and body without pulling, check for a nuchal cord, dry and warm the infant, and move immediately to the Newly Born protocol.
- For prolapsed cord, elevate hips/knees to chest, relieve pressure from the presenting part with a gloved hand, cover exposed cord with moist saline dressing, and transport emergently.
- For breech presentation, support the presenting parts without pulling and create an airway space at the infant's nose if the head does not deliver.

AEMT

- Perform all EMT care plus IV/IO access and protocol-directed treatment of maternal hypotension or hemorrhage without delaying delivery or transport.
- Prepare neonatal resuscitation equipment and obtain additional resources for prematurity or multiple gestation.

Paramedic

- Perform all prior care plus advanced management of maternal shock, seizure, airway compromise, and complicated delivery.

- After field delivery, administer tranexamic acid for postpartum hemorrhage with shock when indicated; oxytocin is not carried on the Claiborne formulary.

Indications

- Pregnant patient with contractions, rupture of membranes, crowning, urge to push, vaginal discharge or bleeding, or concern for imminent delivery.
- Patient who has delivered immediately before EMS arrival and requires maternal or neonatal care.

Assessment | Priority findings

- Crowning or an uncontrollable urge to push.
- Gestation less than 36 weeks.
- Abnormal presentation, prolapsed cord, severe bleeding, seizure, or multiple gestation.
- History of rapid prior deliveries or very frequent contractions.

Assessment | Required documentation

- Contraction onset, duration, and frequency; membrane rupture; delivery time; placenta delivery; maternal bleeding; and treatment.
- Infant APGAR at 1 and 5 minutes without interrupting resuscitation.

Treatment steps

- Delay cord clamping for at least 60 seconds in a preterm infant under 37 weeks who does not require immediate resuscitation.
- Clamp the cord approximately 10 cm from the infant and place the second clamp approximately 5 cm farther away.
- After placental delivery, perform uterine massage for postpartum hemorrhage and continue maternal reassessment.

Medications

Tranexamic acid

1 g IV/IO over 10 minutes for postpartum hemorrhage with shock when within 3 hours of delivery.

- Use with AM-05 Hypotension/Shock and AO-03 OB/GYN Emergency.

Warnings / precautions

- Do not perform a digital vaginal examination.
- Do not pull on the infant, umbilical cord, breech-presenting parts, or placenta.
- Uterine massage is performed only after delivery of the placenta.
- Large-volume or free vaginal bleeding is abnormal and requires urgent hemorrhage management.

Clinical pearls

- Transport is generally preferred, but crowning, urge to push, frequent contractions, parity, and prior rapid labor may make field delivery safer.
- Twins are commonly premature and at higher risk for hypothermia; request additional resources early.

Special populations | Postpartum hemorrhage

- After placental delivery, perform uterine massage for excessive bleeding. For shock, use AM-05 and tranexamic acid 1 g IV/IO over 10 minutes when within 3 hours of delivery.

Source and references

Claiborne County EMS approved protocol manual. | Claiborne County EMS medication formulary, linked protocol pathways, and agency release guidance.

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